Patient Name	MR#	Observation Date	Last Edited Date

SURGICAL PATHOLOGY CASE: Received Date:

Diagnosis
Lymph node, level X, excision:
- No evidence of malignancy (0/1).

Lymph node, level IV-R, excision:
- Positive for metastatic squamous cell carcinoma (1/1).

Lymph node, level VIII, excision:
- No evidence of malignancy.

Lymph node, level IX, excision:
- No evidence of malignancy.

Lymph node, level VII, excision:
- Fragments of benign lymph node.
- Multiple fragments of necrotic tissue.

Soft tissue, (decorticate peel), excision:
- Crystalline material with extensive foreign body response.

Lung, bronchial margin, excision:
- No evidence of malignancy.

Lung, right upper lobe, lobectomy:
- Squamous cell carcinoma, moderately differentiated.
- Tumor measures 3.4 cm in greatest dimension.
- Staple margins free of tumor.
- There is one peribronchial lymph node with metastatic squamous cell carcinoma (1/1).
- Extensive pleural crystalline deposition with foreign body reaction.
- pT2, N2.

Lymph node, level II, excision:
- Lymph node free of tumor (0/1).

(Electronically signed by) Verified

Clinical Information
The patient is a year old female with a lung mass.

Frozen Section Diagnosis
GFS1: Lung, bronchial margin, excision:
- Negative for malignancy.

HFS1: Lung, right upper lobe, lobectomy:
- Squamous cell carcinoma.
(Both per Dr.

Gross Description
The specimen is received in nine separate containers labeled with the patient's name and medical record number.

The first specimen is further labeled "level X lymph node". It consists of a single

fragment of blue-black soft tissue measuring 0.6 x 0.4 x 0.2 cm. The specimen is submitted entirely.

The second specimen is labeled "level IV-R lymph node". It consists of multiple fragments of tan and blue-black soft tissue measuring in aggregate 3.3 x 2.5 x 0.8 cm. The specimen is serially sectioned and submitted entirely in cassettes B1-B2.

The third specimen is labeled "level VIII lymph node". It consists of three fragments of blue-black soft tissue measuring in aggregate 1.7 x 1.5 x 0.7 cm. The specimen is submitted entirely.

The fourth specimen is labeled "level IX lymph node". It consists of a minute fragment of pink soft tissue measuring 0.3 x 0.2 x 0.2 cm. The specimen is submitted entirely.

The fifth specimen is labeled "level VII lymph node". It consists of four fragments of pink and blue-black soft tissue measuring in aggregate 2.0 x 2.3 x 1.2 cm. Each fragment is bisected and submitted entirely.

The sixth specimen is labeled "decorticate peel". It consists of a single fragment of tan-pink soft tissue measuring 1.3 x 0.4 x 0.4 cm. The specimen is submitted entirely.

The seventh specimen is labeled "bronchial margin". It consists of a single segment of pink firm tissue measuring 0.6 x 0.5 x 0.2 cm. The specimen is submitted entirely for frozen section diagnosis and is now resubmitted for permanent section.

The eighth specimen is received fresh for frozen section labeled with the patient's name, medical record number and designated, "right upper lobe". The specimen consist of a fragment of lung tissue which measures 10.5 x 8.7 x 3.4 cm. The external surface of the lung shows a fixed fibrinous deposit along one aspect. There are multiple nodules grossly visible. The largest of which measures 2.4 x 1.6 x approximately 1.7 cm. A section of the largest nodule was submitted for frozen section and is now resubmitted in its entirety for permanent sections in cassette H1. This mass appears to be within less than 1.0 mm of one of the two stapled margins that is identified. There is a second nodular area located within 2.0 cm of this first nodule which measures 2.5 x 1.0 x 0.3 cm. The stapled margins are removed and the lung is serially sectioned revealing that the larger nodule is in communication with a cavity within the pulmonary parenchyma. This cavity contains apparently necrotic tumor. Upon sectioning, it becomes apparent that the solid nodule is larger than previously estimated with its greatest dimension being approximately 3.4 cm. This nodule lies adjacent to lymph nodes which is sectioned revealing a white and gray somewhat gritty cut surface. Representative sections of the specimen are submitted.

The ninth specimen is labeled "level II lymph node". It consists of three fragments of tan soft tissue measuring in aggregate 1.7 x 1.3 x 0.6 cm. Each fragment is bisected and
GENERAL

Block Summary

A1-	Level X lymph node.
B1-B2-	Level IV-R lymph node.
C1-	Level VIII lymph node.
D1-	Level IX lymph node.
E1-E3-	Level VII lymph nodes.
F1-	Decorticate peel.
G1-	Bronchial margin.
H1-	Section through tumor.
H2-	Stapled margin.
H3-	Stapled margin.
H4-	Stapled margin.
H5-	Necrotic debris within cystic space.
H6-H7	One lymph node in five sections.
H8-	Two sections through the larger nodule.
H9-	Sections showing fibrinous nodularity on external lung surface.
H10-	A section of grossly normal lung.

I1-I2- Level II lymph node.

Microscopic Description
A microscopic examination has been performed.
SP Synoptic Report
H: Thorax Lung

MACRO
SPECIMEN:
Lobe(s) of lung: right upper lobe
PROCEDURE:
Lobectomy
SPECIMEN INTEGRITY:
Indeterminate
SPECIMEN LATERALITY:
Right
TUMOR SITE:
Upper lobe
TUMOR SIZE:
Greatest dimension: 3.4 cm
TUMOR FOCALITY:

Cannot be determined

MICRO
HISTOLOGIC TYPE:
Squamous cell carcinoma
HISTOLOGIC GRADE:
G2: Moderately differentiated
VISCERAL PLEURA INVASION:
Not identified
TUMOR EXTENSION:
Not identified
BRONCHIAL MARGIN:
Uninvolved by invasive carcinoma
VASCULAR MARGIN:
Cannot be assessed
PARENCHYMAL MARGIN:
Uninvolved by invasive carcinoma
PARIETAL PLEURAL MARGIN:
Uninvolved by invasive carcinoma
CHEST WALL MARGIN:
Cannot be assessed
TREATMENT EFFECT:
Not applicable
LYMPH-VASCULAR INVASION:
Present
LYMPH NODES:
Present
TNM DESCRIPTORS:
Regional Lymph Nodes (pN)
PRIMARY TUMOR (pT):
pT2a: Tumor > 3 cm, and < 5 cm in greatest dimension surrounded by lung or visceral pleura, without bronchoscopic evidence of invasion in the main bronchus; or Tumor < 5 cm in greatest dimension with any of the following: involves main bronchus, 2 cm or more distal to the carina; invades the visceral pleura; associated with atelectasis or bstructive pneumonitis that extends to the hilar region
REGIONAL LYMPH NODES (pN):
pN1: Metastasis in ipsilateral peribronchial and/or ipsilateral hilar lymph nodes, and intrapulmonary nodes, including involvement by direct extension
DISTANT METASTASIS (pM):
Not applicable